KEY BENEFIT ADMINISTRATORS

SOUTHWESTERN HEALTH RESOURCES

DTE EXECUTIVE SUMMARY

OPPORTUNITY REVIEW

Prepared by Derek Moore
SVP, Hospital DTE Relationships
September 26, 2025

EXECUTIVE LETTER

Dear Southwestern Health Resources Leadership Team,

On behalf of Key Benefit Administrators (KBA), I am pleased to present this proposed partnership executive summary. With almost 50 years of experience, KBA today is the nation's largest, privately-held TPA delivering a fully integrated stack of health plan management capabilities. We are proud of our track record of providing exceptional health plan management to the self-funded industry, helping health systems successfully launch, grow and manage direct-to-employer (DTE) health plans since 2016.

At KBA, we are committed to being part of the solution for systemic healthcare challenges. Our approach centers on transparent plan administration, ensuring benefits are completely free from 'shared savings,' 'spread pricing,' and other practices that divert money away from patient care. We also believe integrity and core principles are central to strong healthcare systems, which is why KBA has never participated in reference-based pricing or similar practices.

Our decade of DTE experience has taught us that having the right market partner is critical to success. We actively market and sell our partners' healthcare services, understanding that their reputation forms the foundation of health plan success. For this reason, we carefully research and vet every potential partner, often turning down health systems that lack the necessary market reputation or network access.

Southwestern Health Resources (SWHR) stands apart as one of the very best positioned health systems in the country to support its own private-labeled DTE health plan. KBA would be honored to partner with SWHR and is confident in the plan's success. We look forward to discussing how our partnership can deliver exceptional value to the employers in DFW and their employees you serve.

Sincerely,

Derek Moore
SVP, Hospital DTE Relationships
Key Benefit Administrators

WHY KBA?

KEY BENEFIT ADMINISTRATORS

- Founded in 1979, headquartered in Indianapolis
- Largest Independently-Owned Full-Service Third-Party Administrator
- Specialize in Administration of Self-Funded Plans and Population Health Management
- Leader in the Direct-to-Employer Movement
- We Know and Understand the Market
- Experience Working with All Professional Teams: Agents, Networks, MGAs
- The Key Family of Companies is a Vertically Integrated group of benefit related organizations
    - 3,000+ Corporate Clients
    - 3 Million Members under Management
    - 500+ Employees

MARKET LEADER IN PRIVATE LABELED DTE PLANS

CUTTING OUT THE MIDDLEMAN

PUTTING A NAME TO THE PROBLEM

CHANNEL DEPENDENCY RISK

PORTER'S FIVE FORCES

Channel dependency risk is a form of bargaining power of suppliers.

EXAMPLES OF CHANNEL DEPENDENCY RISK

Amazon hurts publishing houses

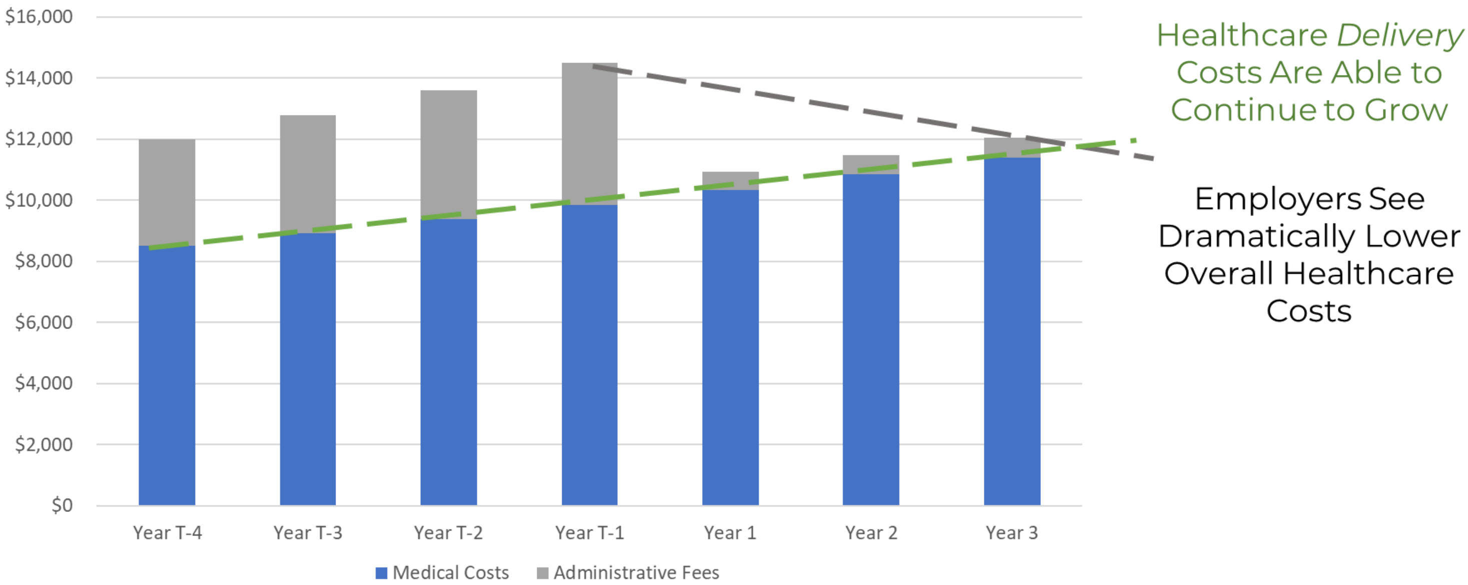
App stores hurt mobile apps

Gig economy platforms hurt gig workers

Netflix hurts film studios

Healthcare's middlemen, insurance companies, were initially a (Texas) healthcare innovation to ensure healthcare reimbursement.

Today, healthcare payers often abuse their position in ways that cause adverse health events and higher costs.



KBA enables direct to employer networks, helping healthcare providers **retake** a critical go-to-market channel and **regain** control over their current and future care delivery.

KEY BENEFIT ADMINISTRATORS

OUR 'WHY': THE PHOTO MOMENT

THE KBA REWARD THROUGH SEEING A DIFFERENCE

Better Care, Lower Costs, Real Partnerships

- All Stakeholders at the Table (Employer, Provider, KBA, Broker/Consultant)
- Reviewing Real Progress (Improved Population Health and Lower Spend)
- Deep Loyalty Between Employer and Provider Organization Built on Value

The RED LINE = The year over year healthcare component of the consumer price index

The BLUE LINE = What happens when you manage your health on Main Street

American Health Data Institute

For our clients, 98% of our Blue Lines are BELOW their Red Lines!

American Health Data Institute

Financial Savings & Return On Investment

with the Health Care Nurse Navigator Chronic Disease Management Program

AHDI has five different statistically validated ROI studies performed in 2006, 2008, 2010, 2014 and 2021. All studies support a consistent and increasing reduction of medical spending between 11-17%.

2021 CDM Analysis Once Again Validates the Value of AHDI's CDM Program

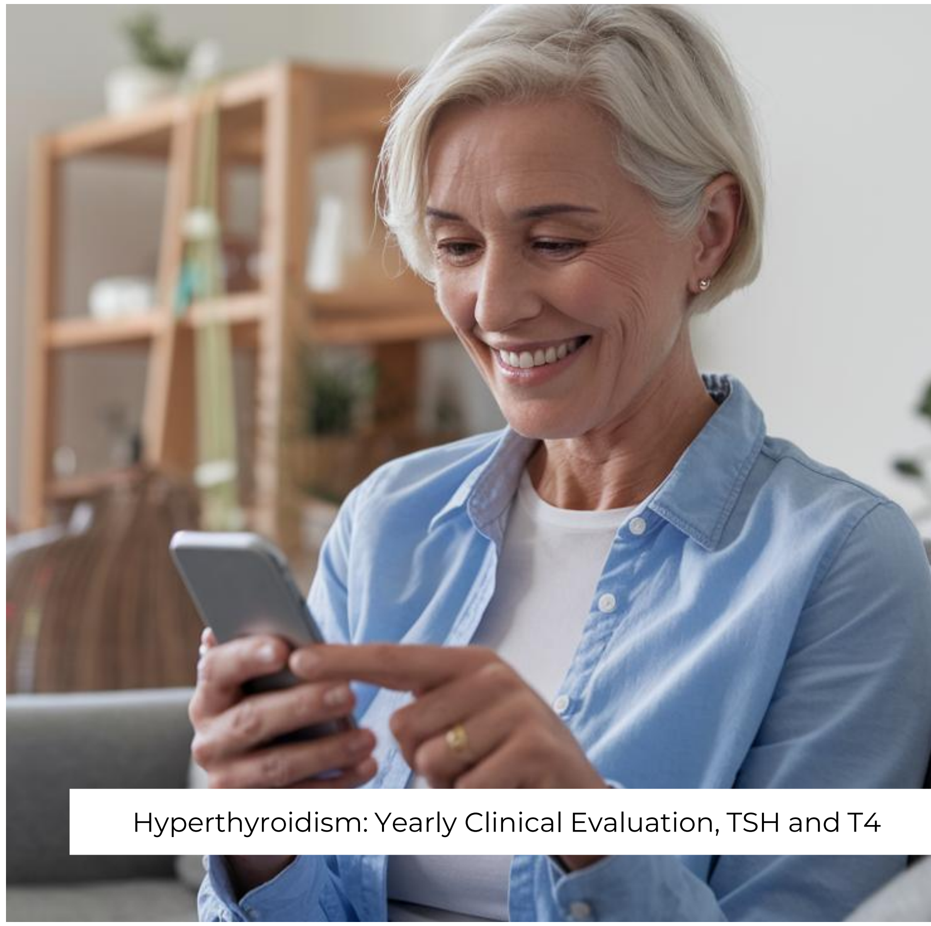
Summary:
This study used claims on 100,000 plus CDM members with continuous enrollment over a two-year period.

Discussion:
A fixed effects least squares regression model was used in this analysis. The use of a fixed effects model controls for price variation between healthcare markets.

Findings:
Participation in the AHDI Chronic Disease Management program was associated with a continued average reduction in claims cost of 11% to 17%, at the 90% confidence interval. This results in an average Return on Investment of 14 to 1. These results are consistent with those of the prior five studies conducted on the CDM program.

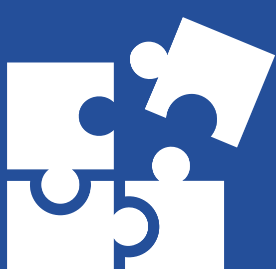
PARTNERING FOR POPULATION HEALTH

KBA shares evidence-based standards of care for 28 chronic conditions directly into Epic



Hyperthyroidism: Yearly Clinical Evaluation, TSH and T4

THE MISSING PIECE TO VALUE-BASED CARE



- Patient engagement is the weak link in delivering value-based care; SWHR's clinical leadership and employer participation can drive better outcomes.
- KBA's proactive care-management team supplements your care-management efforts in outreach to plan members.
- SWHR excels in Standards of Care completion rates for the Chronically Ill, improving care and costs.
- Integrating SWHR's managed care expertise with KBA's population health management programs positions SWHR as the value leader.

COMPLETE NETWORK MANAGMENT

Full-Service Network Management

Network Analysis and Modeling

- Analysis of member access in each sub-geography and for different care journeys
- Evaluations of quality, efficiency and cost effectiveness of providers within the network
- Presentation of pricing and network completeness to funding partners and employer groups

Contracting and Engagement

- Full-service network management services to expand network access through contracting with independent providers
- Strong collaboration and frequent engagement with SWHR to ensure network is optimized with SWHR's consent
- Management of contract negotiations with non-SWHR groups

Operational Success

- Delegated credentialling to SWHR
- Management of provider demographic data and fee schedules
- Accurate and timely provider payments

TURNKEY SALES AND MARKETING

COMMUNITY BASED SALES STRATEGY

Locally based sales rep with strong community ties

Market research and insights to identify strategic employers for early adoption and sales

Work with select broker distribution partners with aligned incentives and goals

Pipeline reporting and collaboration with our DTE provider partners

COMPLETE MARKETING SYSTEM

- Customized Branding and Messaging
- Customized Brochures
- Customized Web Site
- Targeted Employer Outreach
- Sales System (Social Media, Email, Text, etc.)

YOU Deliver Exceptional Care

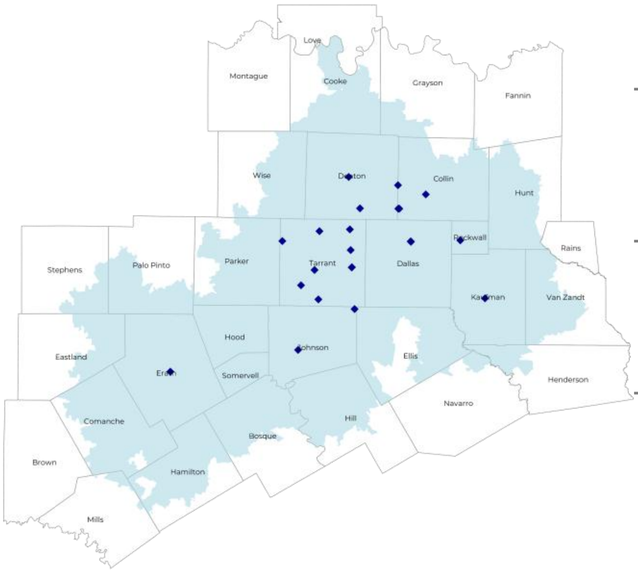
YOU Control the Plan

WE Successfully Grow and Administer

KEY BENEFIT ADMINISTRATORS

180 DAY SALES PLAN

12,000+ TARGET LOCAL EMPLOYERS



	Employer Size Group (# of Employees)					
Commercial Employers With Headquarters In Catchment Area	**50-100**	**101-200**	**201-400**	**401-1,000**	**1,001-3,000**	**3,000+**
	6,061	2,761	1,835	1,021	369	302
Total Employees at Employers Headquartered in Catchment Area	**50-100**	**101-200**	**201-400**	**401-1,000**	**1,001-3,000**	**3,000+**
	454,600	414,200	550,500	714,700	738,000	1,812,000
Total Members with Employers Headquartered in Catchment Area	**50-100**	**101-200**	**201-400**	**401-1,000**	**1,001-3,000**	**3,000+**
	818,300	745,600	990,900	1,286,500	1,328,400	3,261,600

180 DAY SALES TARGETS AND PLAN

Team in place (2 local sales leaders)

Operations Running Reporting, Check-ins Etc.

Day 1-30

- Introduce SVP and VP of Sales to provider management team
- Engage recruiter and local contacts to hire 2 local salespeople
- Coordinate meeting with provider marketing and public relations teams
- Research and do local reconnaissance of the broker community. Determine top broker candidates to help sell the health plan

Day 30-60

- Begin interview process for 2 local salespeople
- Engage key brokers and begin to have meetings to explain the health plan value proposition
- Begin to outline target list of employers
- Begin to outline joint marketing and PR plan
- Engage PR firm

Day 61-90

- Hire 2 local salespeople
- Enroll salespeople in KBA training program (4 weeks)
- Refine employer target list
- Construct digital sales campaign and cadence for digital engagement direct to employers

Day 91-120

- Salespeople are in local community
- Continue broker outreach and engagement
- Begin to execute joint marketing plan (provider and health plan)
- Determine date, place and invite list for broker and employer seminar(s)
- Construct digital and paper invitations for seminar(s)
- Start digital sales campaign

Day 121-150

- Begin to execute PR plan
- Outline co-marketing initiative
- Distribute seminar(s) invitations
- Hold DTE seminar(s)
- Lead follow up from seminar(s) and digital sales campaign

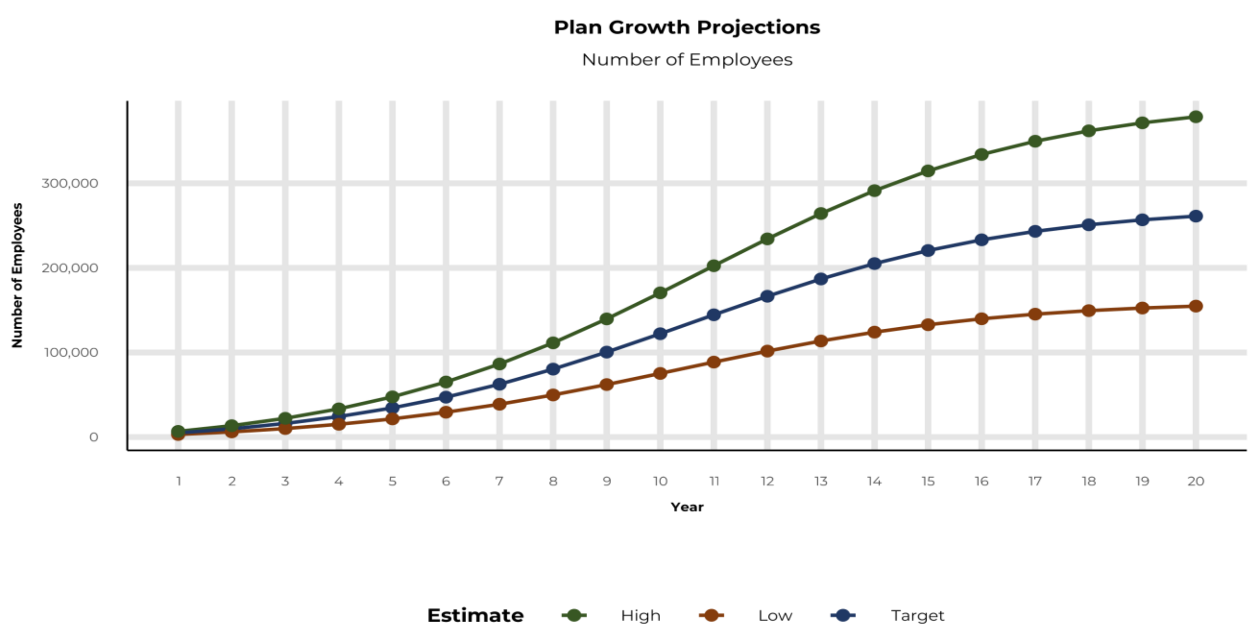
Day 151-180

- Start second tier sales campaign
- Set date, time and cadence with provider leadership for pipeline review
- Begin to conduct agency (broker) value proposition training

PREMIUM HEALTHCARE BRANDS

Health System	HCAHPS Recommendation Percentile	CMS Stars Percentile	Mortality Percentile	Readmission Percentile	KBA Brand Quality Score
University of Texas System	**94**	**100**	**63**	**57**	**75**
Methodist Health System	69	62	66	66	70
Baylor Scott & White Health	72	86	69	67	70
Texas Health Resources	**64**	**73**	**70**	**75**	**67**
Parkland Health	55	31	64	62	64
HCA Healthcare	45	57	86	57	60
John Peter Smith Hospital	33	61	52	65	57
Other Independent Hospitals	30	45	60	57	56
Prime Healthcare Services	11	87	69	58	47
Heights Healthcare of Texas	4	10	59	60	45

GENERATE PREMIUM PLAN GROWTH



PROJECTED GROWTH DETAILS

Year	Employees	Members	Percent of Members Choosing SWHR for Inpatient Care	Percent of Members Choosing SWHR for Outpatient Care
1	4,900	8,800	70%	60%
2	9,800	17,600	70%	60%
3	16,100	29,000	75%	65%
4	24,200	43,600	77%	68%
5	34,500	62,100	77%	69%
6	47,200	85,000	77%	70%
7	62,500	112,500	78%	71%
8	80,300	144,500	78%	71%
9	100,400	180,700	78%	72%
10	122,200	220,000	79%	72%
11	144,500	260,100	79%	73%
12	166,500	299,700	79%	73%
13	186,900	336,400	80%	74%
14	205,100	369,200	80%	74%
15	220,500	396,900	80%	75%
16	233,200	419,800	80%	75%
17	243,200	437,800	81%	76%
18	250,900	451,600	81%	76%
19	256,800	462,200	81%	76%
20	261,200	470,200	81%	76%

Target Member Growth				
Metric	**Year 1**	**Year 2**	**Year 3**	**Year 4**
Number of Members	8,800	17,600	29,000	43,600
Network Access Fee	161.7K	547.6K	957.2K	1,744.0K
GLP-1 Script Revenue*	72.8K	246.4K	430.7K	784.8K
Implementation Fee (One-Time)	-225.0K	0.0K	0.0K	0.0K
Sales and Marketing Costs	-380.0K	-380.0K	-380.0K	-380.0K
SWHR Net Cash Flow	**-295K**	**414K**	**1,008K**	**2,148.8**
SWHR Members Incremental Margin	**15.8M**	**31.6M**	**57M**	**89.6M**

*Assuming 40% of population is obese and 50% of those utilize GLP-1 reduced-cost scripts

COSTS AND PROFITABILITY

Plan Set-Up Fees

Plan and network creation, registration, marketing, sales, branding and launch.

Total Network Launch Cost	**780K**
SWHR Launch Costs	$225K (9% Reduction)
KBA Launch Costs	**555K**

SWHR launch costs cover employer marketing campaign, stop-loss relationship campaign, and employer communication campaigns.

KBA yearly investment covers network, launch team, project management, technical launch, and network leadership.

Yearly Sales Fees

Two dedicated sales leaders and ongoing sales/marketing support.

Total Yearly Sales, Marketing and Administration Costs	**840K**
SWHR Yearly Investment	$380K (21% Reduction)
KBA Yearly Investment	**460K**

SWHR investment covers part of sales team members costs.

KBA yearly investment covers sales leadership, marketing, website, promotions, network management, patient convenience cards, Key Incentive Plan administration, EHR integration, administrative reporting.

ROI 18 MONTHS AFTER LAUNCH

	SWHR Investment	Total Employees	Monthly Network Access Fee	GLP-1 Revenue	SWHR Revenue	Cumulative Costs/Revenue
Year T-1 Q2	(225K)					(225K)
Year 1 Q1	(95K)					(320K)
Year 1 Q2	(95K)	1450	26K	12K	50K	(365K)
Year 1 Q3	(95K)	2950	53K	24K	102K	(358K)
Year 1 Q4	(95K)	4450	80K	36K	153K	(300K)
Year 2 Q1	(95K)	5730	103K	46K	198K	(198K)
Year 2 Q2	(95K)	6975	126K	56K	241K	(52K)
Year 2 Q3	**(95K)**	**8220**	**148K**	**67K**	**284K**	**137K**
Year 2 Q4	(95K)	9465	171K	77K	328K	369K
Year 3 Q1	(95K)	11000	198K	89K	380K	654K
Year 3 Q2	(95K)	12530	226K	101K	432K	991K
Year 3 Q3	(95K)	14060	253K	114K	485K	1,381K
Year 3 Q4	(95K)	15590	281K	126K	538K	1,824K

REVENUE TO HEALTH SYSTEM PARTNERS

Early Stage

Gaining Name Recognition, Early Growth

New Revenue: **$181.21M**

New Profit: **$47.05M**

Growth Stage

Strong Reputation, Strong Growth

New Revenue: **$1049.77M**

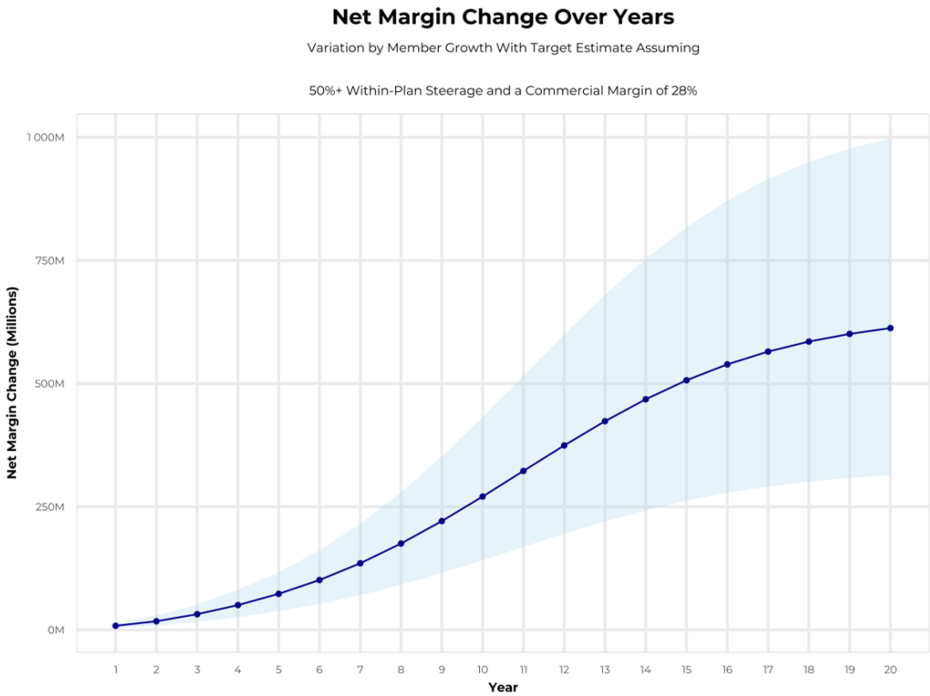
New Profit: **$274.68M**

Mature Stage

High Profitability, Market Saturation

New Revenue: **$2117.2M**

New Profit: **$553.94M**



YEAR 4 MODELED PARTNER BENEFITS

Pre-DTE Cohort of 43,600 Members	Inpatient	Outpatient	Professional	Total
Revenue†	$ 26,975,483	$ 23,078,595	$ 10,777,508	$ 60,831,586
Net Income‡	$ 6,905,724	$ 5,908,120	$ 2,759,042	$ 15,572,886

With DTE	Inpatient	Outpatient	Professional	Total
Revenue⁰	$ 121,563,337	$ 195,885,195	$ 91,476,721	$ 408,925,253
Net Income*	$ 31,120,214	$ 50,146,610	$ 23,418,041	$ 104,684,865

Total Increase in Net Income	$ 89,111,979
Network Access Fee (Revenue) of $6 PEPM	$ 1,744,000
Total Profit Growth	$ 89,127,979
Minus Implementation Fee (One-Time)	-$ 0
Minus Sales and Marketing (Annual – Ongoing)	-$ 480,000
Total	**$ 90,375,979**

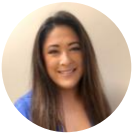
† Assuming proportional revenues from market shares of 17% (inpatient) and 8% (outpatient)

‡ Conservatively assuming a commercial margin of 25.6% (80% of the estimated commercial margin)

⁰ Assuming 77% inpatient and 68% market steerage based on KBA DTE network experience

* Conservatively assuming a commercial margin of 25.6%, with a 10 percentage-point reduction to reflect competitive-edge rates and a 10 percentage-point increase due to lower billing costs and higher patient self-pay collections because of the convenience card.

IMPLEMENTATION TEAM AND TIMELINE



Christine Vinesett

Director, Implementation and Network
I joined Key Benefit Administrators in 2000 and have been in the Health Insurance Industry for 20+ years. My focus has been Leadership in Operations, Carrier and Provider Network Relations and Implementation Management.

Office: (803) 323-7974 | Email: Christine.Vinesett@keybenefit.com

Julie Caskey

Network Operations Lead

Julie Caskey joined Key Benefit Administrators (KBA) in 2021. Julie brings an extensive background with 30+ years in the Health Insurance Industry. Julie's focus has been in Provider Network and Relationship Management.

Office: 317-712-4003 | Email: Julie.Caskey@keybenefit.com

Dejanetta (Deja) Pettaway

DTE Implementation and Relationship Coordinator

Deja joined Key Benefit Administrators in 2023 and has been in the Health Insurance Industry for 10 years. Deja's focus has been in Provider Network and Relationship Management.

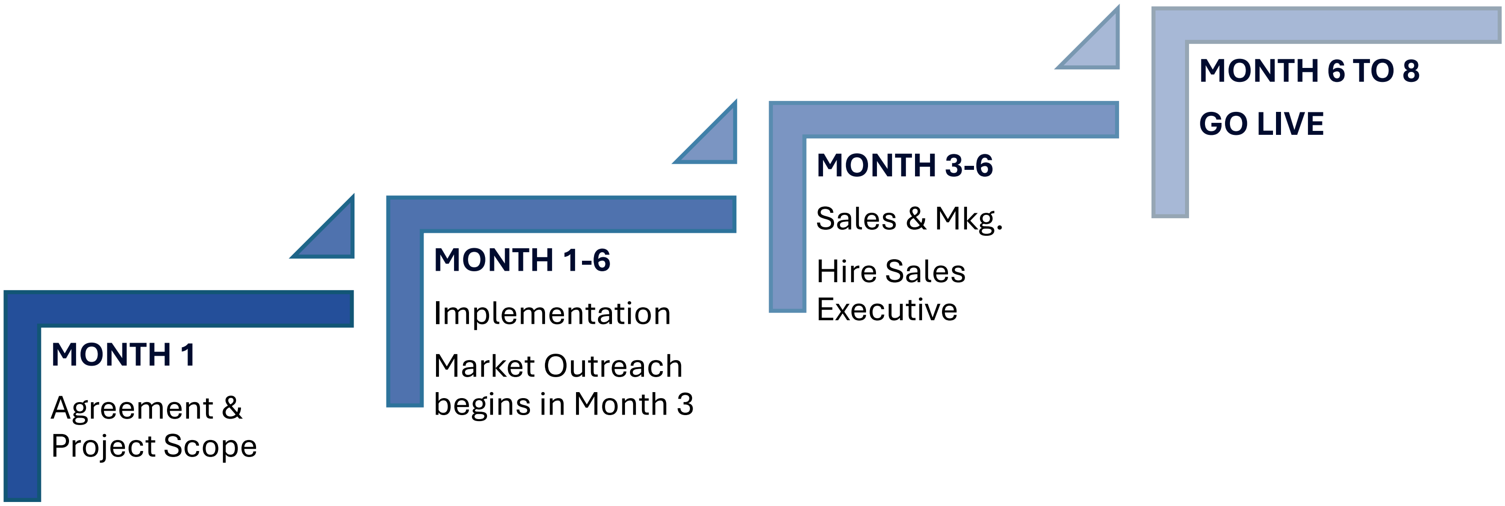
Office: 803-396-4836 | Email: Dejanetta.Pettaway@keybenefit.com

Darlene Grindstaff

DTE Implementation and Relationship Coordinator

Darlene Grindstaff joined Key Benefit Administrators in 2014 and has been in the Health Insurance Industry for 11 years. Darlene's focus has been in Implementation, Provider Network, and Relationship Management.

Office: 803-396-4784 | Email: Darlene.Grindstaff@keybenefit.com



KBA TOTAL QUALITY METRICS

300+	50,000+	$200M+
TOTAL QUALITY MEASURES	UNIQUE AUDITS ANNUALLY	PREMIUM & CLAIMS REVIEWED ANNUALLY

New Business / Renewals

	2023 KBA	2024 KBA	INDUSTRY	GOAL
New Case Accuracy	99.86%	100.00% ↑	N/A	99.75%
Renewal Case Accuracy	99.96%	100.00% ↑	N/A	99.75%
Fulfillment Materials Audit	99.99%	99.98% ↓	N/A	100%

Enrollment Center

	2023 KBA	2024 KBA	INDUSTRY	GOAL
Soft Skills Audit	99.43%	99.22% ↓	N/A	98.00%
Procedural Skills Audit	98.26%	98.23% ↓	N/A	97.00%

Billing & Eligibility Quality

	2023 KBA	2024 KBA	INDUSTRY	GOAL
Eligibility Accuracy	100.00%	100.00%	N/A	99.75%
Plans/Rates/Coverage Audits	100.00%	100.00%	N/A	99.75%
Cash Receipts Logging	99.97%	99.78% ↓	N/A	98.00%
Cash Posting Accuracy	99.96%	99.78% ↓	N/A	97.00%

Claims Quality

	2023 KBA	2024 KBA	INDUSTRY	GOAL
Turn Around Time (Days)	3.79	6 ↓	10	7
% Paid within 10 Bus Days	88.90%	87.34% ↓	80.00%	85.00%
Financial Accuracy	99.30%	99.44% ↑	99.00%	99.00%
Procedural Accuracy	99.00%	99.16% ↑	N/A	95.00%
% of Claims Audited	3.77%	3.80% ↑	1%	3%

Customer Experience Quality

	2023 KBA	2024 KBA	INDUSTRY	GOAL
Average Speed to Answer	22 sec	44 sec ↓	<45s	<30s
% Answered w/in 45 Sec	84.90%	75.00% ↓	80-60%	80-30%
Soft Skills Audit	99.50%	99.54% ↑	N/A	98.00%
Procedural Audit	99.22%	99.10% ↓	N/A	97.00%
Email Correspondence Audit	100.00%	100.00%	N/A	98.00%
Web Chat Corresp. Audit	99.64%	99.50% ↓	N/A	98.00%
Abandonment Rate	1.00%	1.78% ↓	<5%	<1%

PREPARED FOR GROWTH

With more than 45 years of delivery excellence, KBA is the best-positioned partner in the market to support rapid-scale growth

Dedicated Large Case Implementation Teams – Specialized onboarding staff with deep expertise in high-volume, high-complexity group transitions.

Specialized EDI Team – Dedicated experts who streamline vendor and data partner integrations, ensuring rapid, accurate, and secure connectivity.

Compliance & Security at Scale – HIPAA-compliant systems to maintain quality and regulatory standards under high volume.

High-Performance Claims Processing – Automation and dedicated processing workflows to handle surges in claims volume without delays.

Proven Capable – Experience implementing and administering plans for groups exceeding *20,000* members, including complex multi-state and multi-product arrangements.

Flexible Infrastructure – Cloud-based platforms that scale processing capacity, data storage, and integrations without disruption.

Configurable and Flexible Plan Designs – Ability to tailor benefits, eligibility rules, and network arrangements for diverse employee populations.

Rapid Direct-to-Employer Network Builds – Ability to design and launch custom networks and branded solutions within 90–120 days.

Data Integration Expertise – Proven ability to ingest, map, and validate large, complex datasets from multiple sources.

KEY BENEFIT
ADMINISTRATORS

YOUR PARTNER TO REVOLUTIONIZE DALLAS FORT-WORTH HEALTHCARE